

Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think> Reasoning process here: The baby's symptoms, such as tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis, are indicative of respiratory distress. The cyanosis responding well to oxygen administration suggests a condition that is causing an imbalance in oxygenation, but not a cardiac issue. The absence of other symptoms such as feeding difficulties or regurgitation makes an esophageal fistula less likely. The presence of gestational diabetes and the fact that the baby was born after the 32nd gestational week increases the likelihood of a condition related to prematurity. </think><answer> A. Neonatal respiratory distress syndrome (NRDS) </answer>

